

ΣΠΙΑΝΙΣ (SCARCITY): WHAT FILLS THE VOID?

*Because a scarcity becomes an opportunity for someone waiting. This is your journalist,
Ishita Sudhir, reporting from the WHO Committee.*

The importance of a tablet of medicine can be often overlooked. Its importance begins when health fails. It can make a difference between possible recovery and uncertainty, life and death. Yet a medicine is more than a tablet. It carries the expectation and hope that it will help in marking a new beginning. The expectation is often fundamental that the legitimacy of the medicine is often taken for granted. But what happens when a medicine is legal, regulated, and genuine but still does not reach the person who needs it? Then comes the role of counterfeit medicines. Counterfeiting is often viewed as a deliberate act of deception. The law

consequently searches for the offender, the counterfeit, and the point of entry at the chain. Yet the pharmaceutical chain has itself become more complicated. The counterfeit no longer exists just outside the legitimate system. This in turn changes the entire nature of the problem. If these counterfeits were simply manufactured outside the law, then stronger enforcement would have appeared to be the most obvious answer. But the counterfeit medicines are not necessarily the beginning of the problem. It is better derived from a 2024 case study examining the pharmaceutical markets in Ethiopia, which has found that the substandard and falsified medicines



were closely connected with the barriers faced in accessing essential medicines. It concluded that stricter regulation and enforcement alone did not adequately address the factors creating demand for them¹. In fact, it is just the point at which the existing weakness in the system becomes visible.

Even international law is starting to recognize this very distinction. The World Health Organization has distinguished between a substandard medical product as something that fails to meet the established quality standards and a falsified product as something that has been deliberately misrepresented in its identity, composition, or source². This change is not simply terminological. It greatly denotes the factor that a medicine can be authorized but still fail to meet the quality that it is required to provide. Therefore, the law faces an issue that cannot be simply solved by only asking about who owns the medicine. The relevance of intellectual property comes into play as pharmaceutical innovation requires protection. Even though international instruments such as the TRIPS agreement recognize this requirement, it does not place it beyond public health considerations. A.7 and 8 of the TRIPS situate intellectual

¹ <https://pubmed.ncbi.nlm.nih.gov/>

² <https://www.who.int/news-room/fact-sheets/detail/substandard-and-falsified-medical-products>

property protection within the broader objectives of social and economic welfare, thereby permitting the members to adopt the measures ³necessary to protect the public health⁴. Meanwhile, the Doha Declaration has affirmed the right of members to use TRIPS flexibilities to protect public health and promote access to medicines⁵. Thus, the point is not that the intellectual property is not irrelevant but that the legal status of a medicine by itself cannot be the answer as to whether that medicine is reaching the patient safely and affordably. The answer to this can be found in the criminal law, where the MEDICRIME Convention treats the manufacture, supply, and trafficking of falsified medicinal products as a conduct that requires criminalization and international cooperation. These measures can be seen as indispensable when falsification is organized across the borders, yet the criminal law simply answers one question as to who is responsible for putting the falsified products into circulation. The public health must answer another question as to what allowed that product to become capable of reaching a patient in the first place.



⁶Even though these theoretical distinctions exist, for an organ transplant recipient this is painfully practical. For them the correct and consistent use of immunosuppressive medicines such as tacrolimus is critical, and substandardization of the same can lead to serious life-threatening consequences. In such circumstances the question is not simply about access to medicines, but rather it becomes about the access to the right medicine with the right quality and a system that is fully capable of guaranteeing it. Therefore, the fight against falsified medicines cannot end with marking the false medicines easier to identify. By

just identifying, a legal system may punish the manufacturer, a regulator may issue an alert, and a border authority may intercept a shipment. But if the genuine medicine remains accessible, affordable, or unreliable in supply, then the reason for the mere existence of the counterfeits remains intact. Thus, the most difficult issue faced is not how to keep falsified medicines out of the system but to make the legitimate system strong enough that the patients do not have to look beyond it in the first place. Because if the gaps still exist, something else will replace it.

⁴ https://www.wto.org/english/docs_e/legal_e/trips_e.htm?

⁵ https://www.wto.org/english/thewto_e/minist_e/min01_e/mindecl_trips_e.htm?

⁶ Source of Pictures: Pinterest